



RFI Response: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

TO: Department of Health and Human Services
Office of the Deputy Secretary and Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology

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Executive Summary

Pair Team is an AI-enabled medical group serving high-needs Medicaid and Medicare beneficiaries. We operate at the intersection of technology and community health, collaborating with partners like OpenAI, Anthropic, ElevenLabs, academic leaders such as Dr. Roxana Daneshjou of Stanford University, and over 100 community-based organizations to build an AI care system that automates operations and delivers proactive support to patients.

Pair Team believes the most efficient and responsible path toward autonomous virtual care begins with AI serving as the virtual front door and primary coordination layer of care. In this model, AI systems proactively engage patients, identify clinical and social needs, sequence actions, and escalate to human clinicians when clinical risk or decision-complexity cross established safety thresholds. This AI-first approach drives superior patient engagement by providing 24/7 accessibility, multilingual support, and hyper-personalized outreach that extends care beyond the constraints of traditional, resource-limited clinics. **By expanding when and how patients can access care, AI-enabled engagement improves prevention and coordination across healthcare and adjacent public programs, alleviates provider shortages, and reduces total cost of care.** However, existing regulatory, reimbursement, legal, and technical frameworks were not designed for continuously learning, agentic AI. Without targeted modernization, these structural barriers will continue to delay the transition to autonomous care and limit the benefits available to patients.

Regulatory Proposals: A Path to Autonomy

In the last year, HHS has taken enormous steps forward that will encourage the implementation of AI systems in healthcare. Three additional regulatory moves will accelerate action and create a path to continuous AI advancement:

Expand TEMPO into a Multi-Stage Autonomy Roadmap: The FDA’s TEMPO pilot provides a strong foundation for modernizing oversight of AI-enabled clinical tools. HHS should encourage the FDA to utilize the TEMPO pilot to validate a graduated transition to autonomy:

- Stage 1: AI agents operate as recommendation assistants within TEMPO, with higher risk actions validated by a human provider. Developers track performance, including rates of human disagreement with AI recommendations on specific tasks.
- Stage 2: Leveraging the performance data and clinical outcomes collected during the pilot, the AI’s scope expands to include the independent initiation of low-risk diagnostic, clinical or administrative tasks under defined guardrails. For instance, agents may order laboratory tests, low-risk imaging and medication refills. All such actions would be constrained by established national clinical frameworks to ensure that AI reinforces evidence-based practice and avoids generating unnecessary tests, treatments, or procedures with limited clinical benefit.
- Stage 3: As safety and performance are demonstrated at scale, these systems transition into autonomous orchestrators capable of managing complex, whole-person care workflows, including more advanced clinical decisions.

Authorize Developer-Led Evaluation Protocols: The FDA has established an important foundation for iterative improvement through Predetermined Change Control Plans (PCCPs). To support the pace and complexity of modern clinical reasoning models and digital assistants, this framework could be extended through a complementary pilot pathway that authorizes developer-led evaluation protocols and benchmarks. Under this approach, the FDA would review and approve a developer’s evaluation methodology including performance targets, safety thresholds, and validation procedures, rather than requiring resubmission for each discrete model change. Once approved, developers could make changes within the bounds of those validated evaluations, such as incorporating new data types, refining outputs, or expanding clinical scope, provided performance against the approved benchmarks is maintained or improved. This approach aligns regulatory oversight with federal investment in shared evaluation infrastructure, ensuring that AI systems are authorized based on demonstrated performance against rigorous, real-world benchmarks.

Reduce Legal Uncertainty: As AI systems increasingly function as a virtual workforce, existing legal frameworks create uncertainty regarding accountability. When clinicians use AI-enabled tools to inform care decisions, how should responsibility and liability be shared across clinicians, health systems, and technology developers to ensure safe, effective, and accountable care? To address this, HHS should articulate a shared-responsibility governance framework for AI-enabled care that clarifies role-specific expectations and links enforcement action and liability to adherence to core practices, including defining intended use, ongoing monitoring, human oversight, and transparency to patients.

Reimbursement: Scaling ACCESS-Style Care Management Across Medicaid and Medicare Advantage

The ACCESS model demonstrates CMS’s leadership in advancing outcome-aligned, engagement-focused care management under a deflationary pricing scheme. As states and Medicare Advantage plans continue to evolve their care coordination strategies, ACCESS offers a valuable template that can be extended across Medicaid and Medicare Advantage through payment, administrative, and technical levers to support broader adoption. Critically, AI-led care management structures should be tied to deflationary

pricing models by setting rates that require productivity gains from automation and be tied to outcomes based pricing that deliver actuarially supported total cost of care impact.

Medicaid: Reinforcing AI-Compatible Economics and Accountability

Under existing authorities, CMS can encourage states to adopt ACCESS-like care models by issuing targeted guidance and embedding expectations directly into state and plan oversight processes. This can include issuing State Medicaid Director Letters (SMDLs), applying SPA review standards, and influencing managed care contract design during MCO RFP cycles to encourage adoption of ACCESS-style payment, accountability, and delivery approaches.

CMS can further accelerate adoption by:

- Encouraging the use of state-directed payments and managed care incentive payments to invest in ACCESS-like care models and enabling infrastructure.
- Promoting the deployment of AI-enabled agents that access appropriate data sources and interact directly with beneficiaries to support Medicaid redeterminations and community engagement requirements as an initial, high-impact opportunity for beneficiary-facing AI.
- Using RHTP as a recurring incentive by tying annual award criteria to the scope and maturity of AI-enabled care delivery and infrastructure, encouraging states to expand and deepen ACCESS-like models over time
- Publishing and operationalizing standardized templates such as model MCO contract language, acceptable outcome measures and reporting expectations, and illustrative payment models (PMPM, performance-based).
- Creating shared measurement, reconciliation, and payment tools to support consistent implementation, evaluation, and scaling of ACCESS-like programs across Medicaid managed care.

Medicare Advantage: Encouraging Adoption Through Incentives and Enablement

While maintaining flexibility for MA plans, CMS can accelerate voluntary adoption of ACCESS-style models by combining payment incentives with administrative and technical support, including:

- Weighting STARS measures toward ACCESS-aligned clinical outcomes and member experience, such as control of A1c, blood pressure, and lipids, where continuous AI-enabled care coordination can measurably improve performance and reduce downstream utilization.
- Incorporating assumptions of increasing efficiency over time into benchmark rate development, reflecting the role of automation in care coordination.
- Offering technical enablement, such as standardized outcome definitions, benchmarking and risk-adjustment support, and shared reporting and reconciliation tools to reduce execution complexity.

By extending ACCESS-style payment, accountability, and operational logic across Medicaid and Medicare Advantage, CMS can encourage scalable, AI-enabled care coordination while reinforcing flexibility, consistency, and sustainability across programs.

Technology: National Open-Source Infrastructure

To bridge the gap between general AI capability and clinical reliability, HHS should invest in national, open-source infrastructure through federal funding to the private sector and academia, creating a shared foundation for all innovators.

Fund Complex, Real World Benchmarks: Current benchmarks are largely static and single-turn. Additionally, they focus narrowly on clinical decision-making rather than care delivery more broadly. HHS should fund the development of evaluation suites that reflect the actual complexity of care:

- **Care Planning Benchmarks:** Evaluating an agent's ability to sequence clinical and social actions (e.g., identifying a rising A1c and simultaneously triggering a transportation benefit).
- **Engagement Benchmarks:** Shifting from single-turn questions to scaled, multi-turn conversational benchmarks. These are essential for evaluating if an agent can maintain context over an extended dialogue to safely and effectively gather information or guide a patient through a complex process.

Modernize Interoperability for Agentic Access: Safe deployment of agentic AI requires new standards driven by ONC to ensure accountability for automated actions. We recommend:

- **Audit Trails:** Requiring all data requests to identify the requester (human, organization, or automated agent).
- **Standardized "On-Behalf-Of" Delegation:** Explicitly linking an AI agent's data access to its supervising clinician or organization.
- **Mandatory Purpose-of-Use Declaration:** Using existing HL7 vocabularies to declare the intent of an automated action at the time of data access.

Conclusion

AI has reached a critical inflection point where it can solve the gaps in access, quality, cost and efficiency for Medicare and Medicaid beneficiaries that have stymied prior efforts. To realize this potential, HHS must lead a coordinated modernization of the healthcare ecosystem. By evolving regulatory pathways to support agentic reasoning, aligning reimbursement with population health outcomes, and investing in national open-source infrastructure for clinical evaluation, HHS can transform AI from a supportive tool into a scalable virtual workforce. Pair Team appreciates the opportunity to contribute to this RFI and welcomes continued partnership with HHS to build a future of radically affordable, high-quality and accessible care for everyone.